

DISCHARGE SUMMARY

Author: E. Vossmere, MD (NPI 1528374960) | Department: Hospital Medicine | Date of discharge: 05/24/2026 | Status: Final

Patient KORVIN MERROW	DOB / Age / Sex 02/18/1964 / 62 y / Male	MRN KM-6427819	FIN KM-2026-051877
Unit / Room 5 West Medical, Room 5W-318	Date of Admission 05/18/2026	Date of Discharge 05/24/2026	Length of Stay 6 days (HD1–HD7)
Attending E. Vossmere, MD	Service Hospital Medicine	Allergies Lisinopril (cough) — ACE-I intolerance	Code Status Full Code
Disposition Home with home health services	Healthcare Proxy Mara Merrow (wife)	Admitting Dx Urinary-source infection / sepsis physiology	Document Discharge Summary (Final)

REASON FOR ADMISSION

62-year-old man with HFrEF, CAD s/p remote PCI/stent (mid-LAD drug-eluting stent, 09/12/2018), CKD stage 3, type 2 diabetes, and polymyalgia rheumatica on chronic prednisone with uncertain taper, admitted 05/18/2026 after approximately three weeks of decline (progressive weakness, poor oral intake, lightheadedness with a near-fall on 05/17, and family-noted evening confusion). He presented with a suspected urinary-source infection and sepsis physiology on a mixed-physiology background (volume depletion and AKI on CKD).

HOSPITAL COURSE

Admitted to Hospital Medicine and started on empiric ceftriaxone with cautious fluids given HFrEF and CKD. Urine culture grew *E. coli* sensitive to ceftriaxone, and antibiotics were narrowed to oral cefpodoxime accordingly (IV ceftriaxone 1 g q24h HD1–HD3, then oral cefpodoxime 200 mg BID from HD4); he defervesced with resolution of the infection and a fall in WBC from 15.6 to 9.4 k/ μ L. AKI on CKD (creatinine 2.62 down to 1.80 mg/dL) resolved to his outpatient baseline (1.6–1.8 mg/dL) with potassium normalizing from 5.1 to 4.4 mmol/L. Cardiorenal medications were held during the AKI, with Nephrology (Dr. Solthar) and Cardiology (Dr. Caldrane) following for a staged, parameter-driven restart; carvedilol was continued cautiously and tolerated from HD4. Endocrinology (Dr. Veylorn) followed the chronic steroid exposure and inpatient glycemic management; the prednisone taper is to be reconciled with Rheumatology (Dr. Halvek) as an outpatient. PT, OT, and nursing documented function below baseline with high fall risk (Morse Fall Scale 65) and a reproducible medication-management deficit, which shaped the home-support and supervision plan below.

Course by problem:

- 1. Urinary-source infection / urosepsis — resolved.** UA with pyuria, positive leukocyte esterase, and bacteriuria on admission; urine culture *E. coli* sensitive to ceftriaxone. Treated with empiric IV ceftriaxone, de-escalated to oral cefpodoxime with clinical resolution (afebrile, WBC normalized). Antimicrobial course completed during the admission; no home antibiotic is continued.
- 2. AKI on CKD stage 3 — resolved to baseline.** Multifactorial, predominantly prerenal (poor intake, infection physiology) with contribution from renal-sensitive home agents. Managed with cautious isotonic fluids and holding of nephroactive medications. Creatinine 2.62 → 2.38 → 2.12 → 1.98 → 1.86 → 1.80 mg/dL; BUN 48 → 29; K 5.1 → 4.4. Underlying CKD stage 3 unchanged. Avoid NSAIDs; renal-dose review of any new outpatient prescription.
- 3. HFrEF / CAD — chronic therapy with staged restart.** No acute coronary syndrome (no ischemic ECG changes, no troponin elevation); telemetry in sinus rhythm without sustained ectopy. Aspirin 81 mg and atorvastatin 40 mg continued throughout. Sacubitril/valsartan, spironolactone, furosemide, and empagliflozin were held during the AKI;

carvedilol was resumed cautiously from HD4 and tolerated. Per Cardiology and Nephrology, restart of the held cardiorenal agents is deferred to coordinated outpatient follow-up and was not reinitiated in hospital (see Discharge Medications and Follow-Up).

4. Type 2 diabetes mellitus. Home basal insulin glargine continued with inpatient adjustment (back to the home 18-unit dose by HD6) plus correctional lispro; glucose trended from 180–260 to 140–250 mg/dL. Metformin ER and empagliflozin held during acute illness/AKI. No new outpatient glucose-lowering agent was started in hospital; the ambulatory regimen is to be reconciled by the PCP and Endocrinology (baseline A1c 7.6–8.2%).

5. Polymyalgia rheumatica / chronic prednisone. Several-year PMR course on chronic steroids with an uncertain pre-admission taper and unverifiable home adherence. Interim non-abrupt inpatient steroid coverage was maintained. Outpatient rheumatology provenance (Dr. Halvek) documents cautious slow-taper intent with instructions to avoid abrupt discontinuation, and does not assert a numeric dose; the most recent dispensed prescription was prednisone 5 mg with "taper per provider instructions" (05/06/2026). No numeric outpatient daily dose is asserted from inpatient sources. Endocrinology noted that relative adrenal insufficiency is plausible but NOT established and requires outpatient testing under non-stressed conditions.

6. Functional and cognitive status / fall risk. PT: ambulates 100–150 ft with a rolling walker and supervision; Morse Fall Scale 65 (high risk); not yet at prior independent baseline or cane-only. OT: reproducible deficits organizing the 19-item regimen (BID/PRN timing errors), recommending a pre-filled weekly pill organizer with family oversight and teach-back for any new or changed medication. Evening confusion improved but slower with multi-step questions. Supervised mobility and medication supervision recommended for the first 5–7 days at home.

7. Other chronic conditions. Hypertension, mixed hyperlipidemia, diabetic peripheral neuropathy, anemia of CKD, steroid-related osteoporosis, class I obesity, chronic GERD, and chronic constipation tendency — continued on home therapy. Obstructive sleep apnea: home CPAP (variable adherence) — continue at home; not a driver of this admission.

DISCHARGE DIAGNOSES

1. [Principal] Urinary tract infection / urosepsis — resolved (E. coli, ceftriaxone-sensitive).
2. [Secondary] Acute kidney injury on chronic kidney disease stage 3 — resolved to baseline.
3. [Secondary] Heart failure with reduced ejection fraction (HFrEF) and coronary artery disease, s/p remote mid-LAD drug-eluting stent (2018) — chronic therapy with staged cardiorenal restart.
4. [Secondary] Essential hypertension.
5. [Secondary] Mixed hyperlipidemia (on high-intensity statin).
6. [Secondary] Type 2 diabetes mellitus — basal insulin continued; oral agents held for restart reconciliation.
7. [Secondary] Diabetic peripheral neuropathy.
8. [Secondary] Polymyalgia rheumatica on chronic prednisone — taper to be reconciled with outpatient Rheumatology.
9. [Secondary] Anemia of chronic kidney disease.
10. [Secondary] Osteoporosis / osteopenia related to chronic steroid exposure.
11. [Secondary] Obstructive sleep apnea — home CPAP (variable adherence).
12. [Secondary] Class I obesity (BMI 30.6).
13. [Secondary] Chronic gastroesophageal reflux disease.
14. [Secondary] Chronic constipation tendency.
15. [Secondary] Functional decline with high fall risk (Morse Fall Scale 65) and medication-management deficit requiring supervision.

CONDITION AT DISCHARGE

Stable and improved. Afebrile with hemodynamics in his low-normal baseline range (systolic 114–130 mmHg), renal function at baseline (creatinine 1.80 mg/dL, K 4.4 mmol/L), oral intake 75–90% of meals, and mentation clearer than

on admission though still slower with multi-step tasks. Ambulating with supervision using a rolling walker; high fall risk. Discharged home with home health services (referral to Keystone HomeCare Services) and family supervision.

DISCHARGE MEDICATIONS

Reconciled at discharge with Inpatient Pharmacy (E. Marlott, PharmD, BCPS) against the verified home regimen, the inpatient MAR, and consultant recommendations. Prescriptions to Harbor Crest Community Pharmacy, 318 Mill Ridge Road, Bridgehollow, PA, (412) 555-0490.

CONTINUE — chronic home medications

Medication	Dose / Route / Frequency	Indication / Instruction
Aspirin	81 mg PO daily	CAD secondary prevention (single-agent; DAPT completed years ago).
Atorvastatin	40 mg PO nightly	CAD / hyperlipidemia (high-intensity statin).
Carvedilol	12.5 mg PO BID with food	HFrEF / CAD beta-blockade; continued cautiously in hospital and tolerated.
Insulin glargine	18 units SC nightly	Type 2 diabetes basal insulin (home dose).
Gabapentin	300 mg PO nightly	Diabetic neuropathy (resumed; monitor for sedation/fall risk).
Pantoprazole	40 mg PO daily	Chronic GERD.
Ferrous sulfate	325 mg PO every other day	Anemia of CKD.
Alendronate	70 mg PO weekly (Sunday AM)	Steroid-related osteoporosis; take upright, fasting, with water.
Calcium carbonate / vitamin D3	600 mg / 400 IU PO BID	Bone health.
Nitroglycerin	0.4 mg SL PRN chest pain	CAD rescue; max 3 doses, call 911 if persistent.
Acetaminophen	650 mg PO q6h PRN	Pain (avoid NSAIDs given CKD).
Polyethylene glycol 3350	17 g PO daily PRN	Bowel regimen.
Senna	8.6 mg PO nightly PRN	Bowel regimen.

HELD at discharge — staged restart as an outpatient (NOT restarted in hospital)

Medication	Dose / Route / Frequency	Indication / Instruction
Sacubitril/valsartan	24/26 mg PO BID — HELD	HFrEF GDMT. Restart at outpatient Cardiology/Nephrology follow-up per renal and BP parameters.
Spironolactone	25 mg PO daily — HELD	HFrEF mineralocorticoid antagonist. Restart deferred; recheck K and renal function first.
Furosemide	40 mg PO daily — HELD	Diuretic. Restart only if volume reaccumulates; reassess at follow-up.
Empagliflozin	10 mg PO daily — HELD	SGLT2 inhibitor (HFrEF/T2DM). Restart when intake and renal function consolidate, per consultants.
Metformin ER	500 mg PO BID — HELD	Type 2 diabetes. Hold pending renal review; restart per PCP/Endocrinology.

Per Cardiology (Dr. Caldrane) and Nephrology (Dr. Solthar): AKI has resolved to baseline, but home physiologic reserve is not established. These agents are to be restarted in a staged, parameter-driven sequence coordinated between Cardiology and Nephrology at the outpatient follow-up visits — not all at once, and not at discharge. Do NOT restart without follow-up labs and clinician instruction.

PREDNISONE — requires outpatient rheumatology reconciliation

Medication	Dose / Route / Frequency	Indication / Instruction
Prednisone (PMR)	Continue current steroid; DO NOT stop abruptly	No numeric inpatient dose asserted — home taper was not verifiable. Most recent outpatient prescription: prednisone 5 mg, "taper per provider instructions" (Dr. Halvek, 05/06/2026). Continue per the active outpatient rheumatology e-prescription / dispensing record; the structured

		taper is to be finalized by Rheumatology (Dr. Halvek), with Endocrinology input, at outpatient follow-up.
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Sick-day guidance provided to the patient and Mrs. Merrow: if Korvin develops a febrile illness, persistent vomiting, or significant dehydration, contact his physicians promptly for stress-dose guidance rather than stopping or self-adjusting the steroid. Relative adrenal insufficiency is plausible but NOT established; outpatient axis testing under non-stressed conditions per Endocrinology.

Antibiotic: urinary-source infection treated to completion in hospital (IV ceftriaxone → oral cefpodoxime); no antibiotic is continued after discharge.

FOLLOW-UP

Primary care to coordinate appointments and integrate specialty recommendations. Windows and clinicians per the discharge plan and Case Management; transportation is being arranged by Case Management with family and insurance-linked options.

Service	Clinician	Window	Purpose
Primary care	Talia Quenor, MD (412) 555-0410	Within 7 days	Overall reconciliation, review of held/continued medications, BP and orthostatic check, glycemic regimen, symptom and weight review.
Nephrology	Iven Solthar, MD	Within 1–2 weeks	CKD/renal recovery; timing of cardiorenal and metformin restart; electrolyte and creatinine monitoring. Avoid NSAIDs.
Cardiology	Maris Caldrane, MD	Within 2 weeks	HFrEF/CAD GDMT restart titration (sacubitril/valsartan, spironolactone, empagliflozin, diuretic), coordinated with Nephrology; BP/orthostatics.
Endocrinology	Nerea Veylorn, MD	Within 2–4 weeks	Diabetes regimen review (A1c 7.6–8.2%); coordinate steroid/taper implications and axis testing with Rheumatology.
Rheumatology	Soren Halvek, MD (412) 555-0520	Within 2–4 weeks	Finalize the PMR prednisone taper and reconcile the current dose against the active e-prescription / pharmacy fill history.

Recommended labs / monitoring:

- Basic metabolic panel (creatinine, BUN, potassium, bicarbonate) within ~1 week, before or at the first cardiorenal restart, then as directed by Nephrology/Cardiology; more frequently after any agent is restarted.
- CBC to follow hemoglobin (anemia of CKD; baseline 10.5–11.5 g/dL).
- Glucose self-monitoring with review of the log at the PCP/Endocrinology visit; A1c per outpatient schedule.
- Daily weights at home; report a gain of >2–3 lb in a day or >5 lb in a week.
- Outpatient draws available at Mercy Vale Outpatient Laboratory — Harbor Crest, 215 Harbor Crest Way, Suite 110, (412) 555-0455.

HOME SERVICES, EQUIPMENT, AND SUPERVISION PLAN

- **Home health nursing:** referral to Keystone HomeCare Services, (412) 555-0631, for early post-discharge medication review/reconciliation, vital-sign and weight monitoring, and teaching. (Medicare eligibility screening initiated by Case Management; confirm agency acceptance and first-visit date at discharge.)
- **Home or outpatient PT/OT:** continuation of therapy per PT/OT; outpatient OT referral for IADL/medication-management retraining if not improved by ~30 days.
- **Durable medical equipment:** rolling walker (preferred over cane for now), bedside commode, and bathroom grab bars; remove loose rugs from walking paths.
- **Medication supervision (OT):** use a pre-filled weekly pill organizer with family oversight; Korvin should not be the sole person responsible for new or changed medications during the early period. Teach-back with family for any new/changed medication, reconfirmed at 24 hours.
- **Supervision for the first 5–7 days:** supervised mobility and a daily family check-in at medication times, including verification that doses were actually taken. Wife Mara Merrow (healthcare proxy/primary caregiver) covers most days; daughter Lenora

Merrow-Halsey covers weekend daytime and transportation. Arrange coverage for weekday mornings, which the family identified as a gap.

- **CPAP:** continue home CPAP nightly.

PATIENT AND FAMILY INSTRUCTIONS

- Heart and kidney medications were explained together: aspirin, atorvastatin, and carvedilol are continued; sacubitril/valsartan, spironolactone, furosemide, empagliflozin, and metformin were held during the kidney injury and will be restarted one at a time by Cardiology and Nephrology at follow-up, only after repeat labs — do not restart these on your own.
- Keep taking prednisone as currently prescribed; do not stop it suddenly. Bring the prednisone bottle to the rheumatology visit so the exact dose and taper can be confirmed.
- Use the rolling walker and have someone nearby when walking; use the pill organizer and have family confirm doses.
- Avoid anti-inflammatory medications (ibuprofen, naproxen, etc.) because of kidney disease; use acetaminophen for pain.
- Keep all follow-up appointments and complete the recommended lab work.

RETURN PRECAUTIONS

Return to the Emergency Department or call (412) 555-0150 for: fever, worsening weakness, recurrent lightheadedness or a near-fall, chest pain, shortness of breath, orthopnea or rapid weight gain/leg swelling, reduced oral intake, or any new or worsening confusion. For medication or taper questions, contact the PCP (Dr. Quenor, (412) 555-0410) or Rheumatology (Dr. Halvek, (412) 555-0520).

Completed and finalized for discharge on 05/24/2026.

Electronically signed by E. Vossmere, MD — Hospitalist Attending, Hospital Medicine

NPI 1528374960 | Mercy Vale Regional Medical Center | 05/24/2026

Discharge medication reconciliation supported by Ezekiel Marlott, PharmD, BCPS — Inpatient Pharmacy.